

Critical Value and Critical Test Result Reporting

Policy Number	POL-CLN-042	Owner / Department	Laboratory / Medical Staff
Effective Date	2023-09-18	Version	3.0
Last Revised	2026-01-22	Next Review	2028-01-22
Approved By	Patient Safety & Quality Committee	Applies To	Laboratory, radiology, cardiodiagnostics staff and all licensed providers and nurses receiving results

1. Purpose

To ensure that critical results of tests and diagnostic procedures are reported to the responsible licensed caregiver on a timely basis so that the patient can be treated promptly, satisfying The Joint Commission National Patient Safety Goal on reporting critical results (NPSG.02.03.01).

2. Scope

Applies to all critical (life-threatening) values and critical findings from the clinical laboratory, blood bank, radiology, and cardiodiagnostics, whether the result is a first occurrence or a repeat. Each discipline maintains a defined critical-value list approved by the Medical Staff.

3. Definitions

- **Critical value** — a result so far outside the normal range that it represents a life-threatening situation unless action is taken promptly.
- **Read-back** — the receiver repeats the verbally reported result and patient identifiers back to the reporter, who confirms accuracy (closed-loop communication).
- **Responsible licensed caregiver** — the ordering provider or a licensed designee able to act on the result.

4. Procedure — Reporting a Critical Value

1. Verify the result and patient identity before calling.
2. Notify the responsible licensed caregiver by direct verbal contact (in person or by phone) — never by voicemail, text, or an unmonitored queue alone.
3. Use two patient identifiers and state the test, value, and units.
4. Obtain a read-back of the result and identifiers and confirm accuracy.
5. Document the result, date/time of the call, the names of the reporter and receiver, and that read-back occurred.
6. If the caregiver cannot be reached, follow the escalation call list until a licensed caregiver is reached.

IMPORTANT — A critical value must reach a licensed caregiver who can act on it within the timeframe in the table below — measured from result verification. If the first contact is not reached, immediately move down the escalation list; a critical value that is not communicated and acted upon is a serious reporting failure under POL-RM-013.

5. Critical-Value Call List and Timeframes

Tier	Contact	Target timeframe
1	Ordering provider (or covering provider).	Within 30 min of verification.
2	Attending / responsible service if Tier 1 not reached in 15 min.	Within 45 min of verification.
3	Rapid Response Team or hospitalist on call if no licensed caregiver reached.	Within 60 min of verification.

Representative critical values include potassium < 2.8 or > 6.0 mmol/L, glucose < 40 or > 500 mg/dL, hemoglobin < 7 g/dL, platelets < 20×10^9 /L, INR > 5.0, troponin above the facility threshold, and radiology findings such as new intracranial hemorrhage or pulmonary embolism.

6. Severity & Reporting

Report breakdowns in critical-result communication per POL-RM-013.

Severity	Example	Notify / timeframe
SEV-1	Critical value not communicated or not acted upon, contributing to death or permanent harm (sentinel event).	Attending, Administrator-on-Call, and Risk Management within 1 hour.
SEV-2	Critical value reported late / to the wrong patient record with major temporary harm or required rescue treatment.	Lab director and Risk Management within 4 hours.
SEV-3	Read-back omitted or late report with no patient harm.	Supervisor within 24 hours via event report.
Near Miss	Result flagged and corrected before reaching the chart; no action taken on erroneous value.	Event report within 24 hours.

7. References

- The Joint Commission, National Patient Safety Goal NPSG.02.03.01 — report critical results of tests and diagnostic procedures on a timely basis.
- Related: POL-CLN-043, POL-RM-013.